



Fiona Stanley Fremantle Hospitals Group

OFFICIAL

Policy and Procedure

Electroconvulsive Therapy

Reference #: FSFH-MENH-POL-0014

Scope

Site	Service/Department/Unit	Disciplines
Fremantle Hospital (FH)	Mental Health Service	Medical, Nursing, Pharmacy

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The information provided in this document is based on its relevance to Fiona Stanley Fremantle Hospital Group (FSFHG) only. Due to differences in context, scope of practice and differences in service delivery, FSFHG does not make claim to its relevance or appropriateness for use within other organisations/sites.

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1. Introduction

Electroconvulsive Therapy (ECT) is a safe and effective treatment for a variety of psychiatric conditions. The administration of ECT involves a multidisciplinary team, with duties and responsibilities delegated throughout the team according to professional scope of practice.

This document standardises the requirements and processes to be followed for the delivery of ECT at Fremantle Hospital, providing a baseline for continuous improvement of site specific ECT practices.

2. Terminology

Electroconvulsive Therapy (ECT)	A treatment involving the application of electric current to specific areas of a person's head to produce a generalised seizure which is modified by general anaesthetic and the administration of a muscle relaxing agent.			
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Thymatron IV™	The machine that delivers ECT treatment, it also provides electroencephalogram (EEG) monitoring of the seizure.
ECT Interventionalist	The person administering the ECT treatment, either: • RANZCP accredited psychiatrist trained in contemporary ECT practice, including the use of EEG monitoring or • a medical practitioner under supervision.
Emergency ECT	ECT given as an emergency procedure to an adult involuntary patient: • To save the patient's life • Where there is imminent risk of the patient behaving in a way that is likely to result in serious injury to the patient or another person.
ECT Lead	Consultant Psychiatrist in charge of ECT

3. Policy

- The Mental Health Service (MHS) will maintain approval under section 544 of the MHA 2014 for the provision of ECT.
- The MHS has an allocated Consultant Psychiatrist in charge of ECT. The Head of Service will assume governance of ECT in the absence of the Consultant Psychiatrist in charge of ECT.
- Consultant Psychiatrists are credentialed with the scope of clinical practice defined to include ECT.
- Psychiatric Registrars rotate through the role of ECT interventionist under the supervision of the rostered Consultant Psychiatrist.
- All ECT administered at FH will comply with the:
 - Mental Health Act (MHA) 2014
 - Chief Psychiatrist's Practice Standards for the Administration of Electroconvulsive Therapy (ECT) in WA 2025.
 - Chief Psychiatrist's Guidelines for the use of Electroconvulsive Therapy in Western Australia 2024
 - Consent to Treatment policy MP 0175/22

- Royal Australian and New Zealand College of Psychiatrists professional practice guidelines for the administration of electroconvulsive therapy 2019

- It is an offence for a person to perform ECT on another person unless in accordance with ss. 194 to 199.
 - Penalty: a fine of \$15 000 and imprisonment for 2 years.
- ECT can only be performed at a mental health service approved under s. 544 for this purpose.

- The MHS does not provide planned ECT to patients aged under 17 years of age.
- Patients will not have a first ECT treatment session on a Public Holiday, unless emergency ECT is indicated and approved by the Head of Service on behalf of the Chief Psychiatrist.
- Patients will be informed of their rights in relation to ECT by their Treating Team and prior to each treatment.
- The ECT service will provide patients and their family/significant others with written and verbal information/explanation regarding ECT in a way that is understandable to them and repeated at intervals throughout the treatment.
- The Interpreter service will be used when indicated.
- The MHS will ensure privacy is maintained for the patient receiving ECT.
- The ECT machine (Thymatron IV™) will be registered on the Australian Register of Therapeutic Devices by Biomedical Engineering.
- The ECT machine will be reviewed annually against current research evidence and updated if it no longer meets best practice requirements.

4. ECT Scheduling

- ECT is only administered in the dedicated ECT suite equipped with appropriate equipment:
 - Patients assessed as high-risk patients may attend the Operating Theatre, B Block FH for treatment and recovery on Monday/Wednesday/Friday following agreement by the ECT Consultant psychiatrist and Consultant Anaesthetist and confirming theatre availability.
- ECT will be routinely administered in the ECT Suite, on Monday's/ Wednesday's and Friday's commencing at 08:00.

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- The service has capacity to provide ECT to a maximum of eight patients per ECT session. Additional cases may be agreed to on an individual basis after discussion between the ECT Consultant Psychiatrist, Consultant Anaesthetist and ECT Coordinator.
- ECT treatment will be provided on public holidays only where clinically indicated. Numbers will be routinely limited to 3 patients with inpatients taking priority.
 - A maximum of 4 patients may be scheduled at the discretion the ECT Consultant Psychiatrist in charge of ECT in liaison with the treating Consultant Psychiatrist, duty Consultant Psychiatrist and Consultant Anaesthetist on the day of ECT.
- The ECT Coordinator will flag high risk/complex patients to the Anaesthetic Consultant / ECT Lead prior to planned ECT treatment to ensure additional anaesthetic staff are available when required.

5. Referral for ECT

- FH accepts referrals for ECT from:
 - FSFHG inpatient wards
 - All Mental Health Hospital in the Home (HiTH) Teams where the patient is suitable for initiation in the community.
 - FH Community Treatment Teams
 - Cockburn Health
 - Patients referred from other areas where there are no ECT services or patient preference due to vicinity.
- All ECT e-referrals need to state the proposed electrode placement. Bitemporal electrode placement referrals need to be discussed with the ECT Lead or Head of Service prior to acceptance. This is due to the increased risk of cognitive side effects and to ensure that bifrontal or right unilateral electrode placements have been considered.

6. Consent to treatment.

- ECT, including maintenance ECT is usually provided as several treatments referred to as a 'course of treatment' and the patient must be made aware that consent is for the course of treatment.

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- Consent must specify the number of treatments in the course of ECT (maximum 12), as determined prior to commencement to the course of treatment. Further consent is required with any specified new course of treatment.
- A patient's consent remains valid until either the patient withdraws it, or the proposed treatment is no longer appropriate due to a change in the patient's circumstances (e.g. change in status under the MHA 2014, treatment options and risks).
- A new consent must also be obtained if, during a course of treatment, there are substantial changes to the treatment (e.g. electrode placement or duration of the course)
- The treating Consultant Psychiatrists must document in the patient's medical record matters which have been discussed including risks explained and questions asked by the patient and the answers given to those questions.
- If a patient requires a further acute course immediately after completing the first course, the ECT Coordinator will escalate to the Consultant Psychiatrist in charge of ECT who will ensure:
 - The patient is discussed at the consultants Peer Review Meeting.
 - A second opinion is obtained if indicated and/or the decision cannot wait until the next Peer Review Meeting

ECT for patients less than 18 years old may only be given with approval by the Mental Health Tribunal.

6.1. Voluntary patient consent

- Must have a valid informed written consent form, which states the:
 - Number of ECT treatments.
 - Total duration of the course of ECT.
 - Position of the treatment electrodes.
 - Details of the guardian/ substitute decision maker providing the consent (where relevant).
 - Signature of third-party witness to the consent.
- A carer, parent or next of kin can only consent for ECT on behalf of a voluntary patient if they are the legal guardian. Where the patient is adamant, they do not want treatment then the Treating Team will:
 - discuss treatment options with the guardian.

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- consider if it is more appropriate to make the patient involuntary (if criteria are met) to provide treatment without consent.
- A copy of the ECT consent must be sighted by the ECT Coordinator and ECT Consultant Psychiatrist prior to the provision of each ECT treatment.
- Consent must be verbally confirmed with the patient prior to every treatment.

6.2. Involuntary patient consent:

- Consent for treatment will be provided via application to the Mental Health Tribunal.
- The Mental Health Tribunal will approve a specified number of ECT treatments.
- Consent must be confirmed prior to every treatment:
 - A copy of the approval for ECT from the Mental Health Tribunal and the current MHA 2014 legal form (Form 6A, 6B or 6C) must be sighted by the ECT Coordinator and ECT Consultant Psychiatrist prior to the provision of ECT.
- Require a Consent to Anaesthesia (General or Regional) form to be completed only if the patient's status under the MHA 2014 changes to voluntary.

6.3. Emergency ECT consent:

- Must be authorised by the Chief Psychiatrist or their delegate (Head of Service) in accordance with s199 of MHA 2014. Refer to [Appendix 1](#) Emergency ECT.
- Approval for ECT must be provided prior to any treatment:
 - A copy of the emergency authorisation by the Chief Psychiatrist or delegate (Head of Service) must be sighted by the ECT Coordinator and ECT Consultant Psychiatrist prior to the provision of ECT.
- A maximum of two emergency ECT treatments may be given before MHT approval is confirmed.

A person under 18 years must not be given **emergency** ECT treatment.

7. Pre ECT-assessment

7.1. Treating team assessment

- A baseline assessment of the patient's symptoms will be documented before ECT, and specific target symptoms will be identified.
- Prior to the commencement of ECT, will complete the following assessments and document in the patient's medical record:

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- Clinical Global Index (CGI)
- Montreal Cognitive Assessment (MoCA)
- Montgomery Asberg Depression Rating Scale (MADRS) where the diagnosis is major depression.
- Brief Psychiatric Rating Scale (BPRS) where the diagnosis is determined to be a psychotic disorder.
- Kessler 10 (K10).

7.2. Anaesthetic assessment

- A pre-ECT anaesthetic assessment must be completed prior to the start of a treatment course, to allow adequate time for further investigations and/or consultation. The assessment will be documented in the patient's medical record.
- The Treating teams will liaise with the ECT team for pre-anaesthetic assessments:
 - For all FSH and FH inpatients, the treating team will contact the Duty Anaesthetist to organise an inpatient anaesthetic review.
 - For all outpatients and Cockburn Health patients, the treating team must inform the ECT team, who will organise an appointment for the patient to attend the ECT suite for anaesthetic review
- The ECT team will arrange for outpatients and Cockburn Health patients to attend the ECT suite at FH on ECT days to complete the anaesthetic review prior to commencing the course.
- The Pre ECT-assessment will include:
 - a thorough history and physical examination.
 - an evaluation of dentition to identify the presence of dentures and/or dental problems that may affect the use of the bite block. Temporal-mandibular joint problems should also be highlighted.
 - an electrocardiogram (ECG).
 - echocardiogram for patients over 65 years if not conducted in previous 12 months.
 - other routine laboratory investigations should be guided by the patient's history and physical examination e.g. full blood examination, urea and electrolytes, psychotropic serum levels, renal function tests and fasting blood glucose.
 - the treating team must organise lithium, valproate, and clozapine levels to be taken for relevant patients. These results must be documented prior to ECT.

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- a chest X-ray is only required if clinically indicated, e.g. patients with an unstable cardiopulmonary condition.
- a cervical spine X-Ray is indicated in patients with suspected cervical spine instability e.g. rheumatoid arthritis and/or severe osteoporosis.
- the pre-anaesthetic check will be reviewed prior to each treatment and updated if there is a change in comorbidities.
- A specialty consult e.g. cardiology is advised where ECT could substantially increase risks due to an existing condition.
- The ECT Coordinator will arrange for outpatients who have not had an anaesthetic review in the previous 12 months to attend the ECT suite at FH on ECT days for an anaesthetic review.

8. Medication

8.1. Prior to commencement of ECT

- A medication review will be conducted prior to starting a course of ECT by the treating team and clinical pharmacist (for inpatients):
 - Outpatient medication reviews by a clinical pharmacist are available via eReferral.
- Medications are to be optimised in preparation for ECT:
 - existing medications for medical illness can usually be continued.
 - consideration should be given to psychotropic medications during a course of ECT.
- Refer to Appendix 2 for specific details on optimisation of individual/class of medications.
 - for inpatients, the clinical pharmacist will annotate on the medication chart if medications are to be withheld, given before ECT or after.
 - for HiTH patients and outpatients, a medication list will be provided to the patient following the medication review, indicating if medications are to be withheld, given before ECT or after.

8.2. During a course of ECT

- Critical medications are to be administered preferably at least two hours before ECT:
 - oral medications are to be given with small sips of water to reduce risk of aspiration during ECT.
- Non-time critical medications will be administered after the treatment when the patient is fully awake.

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- Nurses are to annotate the time on the medication charts when the medications are administered.

9. ECT Equipment

- Prior to the commencement of the ECT list:
 - The Consultant Psychiatrist or Registrar performing ECT and the ECT Coordinator will ensure that the ECT machine is functioning.
 - The anaesthetic team will check anaesthetic monitoring, airway equipment including functioning suction and emergency drugs (as per [ANZCA PG18\(A\) Guideline on monitoring during anaesthesia](#)).
- Anaesthetic equipment must be in place prior to the induction of anaesthesia and includes:
 - ECG
 - Pulse oximeter
 - Non-invasive blood pressure
 - Capnography
- Prior to the arrival of the first ECT patient to the Holding Bay/Recovery Bay recovery staff will check monitoring and resuscitation equipment including defibrillator and suctioning. Refer to the Resuscitation and Emergency Equipment Content Checking and Layout (Adult) Guideline

10. Patient care

10.1. Patient preparation

- Careful skin preparation is essential. Hair and scalp should be clean and as free from skin or hair products as possible.
- Metallic piercings on eyebrows and tongue must be removed prior to ECT.
- Shaving hair is NOT required.

10.2. Clinical handover

- The ECT Nursing Handover Checklist is completed by the:
 - ward nurse escorting the patient to ECT for inpatients.
 - ECT nurse for outpatients on arrival to the suite.
- On arrival in the ECT suite the ward nurse will provide iSoBAR handover to the ECT nurse for inpatients.

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- The ECT Coordinator will crosscheck the ECT Nursing Handover Checklist with the ECT nurse and sign prior to each session.
- ECT Coordinator will obtain verbal consent from the patient and confirm the procedure with the patient prior to proceeding (voluntary patients).
- The MHA 2014 status of every patient must be checked by the ECT Coordinator, the ECT Registrar and the ECT Consultant Psychiatrist prior to the provision of every treatment.

10.3. Treatment

- Ensure the chosen electrode sites are thoroughly cleansed with alcohol-soaked swabs and gauze squares used to dry and remove oil, makeup and previous gel.
- Clean bilateral mastoid area with an alcohol-soaked swab and dry with gauze.
- Cover patient's eyes and ask patient to close their eyes, then spray area of electrode sites with electrode solution skin prep spray.
- Apply a conductive liquid (PreTac™) to the skin where the electrode patch will be applied.
- Press the electrode patch firmly against the skull to minimise impedance.
- Team Time Out is performed immediately prior to treatment with the ECT Coordinator, Psychiatric Registrar, ECT Consultant Psychiatrist, Consultant Anaesthetist confirming the following:
 - patient identification
 - patient Mental Health Act Status
 - electrode position
 - treatment dose
 - impedance
 - anaesthetic dose from previous ECT treatment
- Team Time Out is documented by the ECT Coordinator on the Procedure Safety Checklist and in the Theatre Management System (TMS) each session.
- The starting dose will be determined by the stimulus dose 'titration method' which involves stimulating the patient with a gradually increasing electrical dose, starting at a low dose. Refer to Appendix 3 & 4 for starting doses and up titration.
- If no seizure is noted on either motor and/or electroencephalogram (EEG) activity, another stimulus under the same anaesthetic can be given at a higher dose, after another 30 to 45 second time lapse if no additional anaesthetic and muscle relaxant needs to be given.

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- Maximum of three attempts if no seizure in a single treatment
- The delivery of more than one seizure per treatment session (including a threshold seizure during titration) will not be used.
- Bi-frontal/bi-temporal electrode placement treatment is optimised at 1.5 times the seizure threshold.
- Unilateral electrode placement treatment is optimised at 5-6 times the seizure threshold.
- Where a patient has previously been treated with ECT at FH, the starting dose of both ECT and anaesthetic as detailed in the South Health Information Management System (SHIMS) ECT database will be reviewed.

10.4. Prolonged seizures

Prolonged seizures are a possible complication of ECT. In the ECT Suite a prolonged seizure is defined as seizure activity lasting longer than 120 seconds.

- Seizure activity is monitored both visually and via EEG monitoring.
- In the case of an actual or suspected prolonged seizure, the ECT coordinator will:
 - Alert the anaesthetist if the seizure continues beyond 90 seconds to prepare for termination.
 - At 110 seconds, the seizure will be terminated by the anaesthetist using an anticonvulsant agent such as propofol and/or midazolam and/or intubate if indicated.
 - Keep EEG printout running until seizure activity has stopped completely and returned to baseline.
- A Code Blue will be called when clinically indicated.

10.5. Monitoring

- EEG monitoring is always right and left mastoid and two mid frontal for all electrode placements.
- Effectiveness of a seizure will be measured by the ECT/EEG Rating Scale. The following seizure monitoring will occur throughout the procedure:
 - Duration of the seizure
 - Quality of tracing
 - Maximum-minimum pulse rate ratio
 - Post Ictal Suppression Index.
- The following anaesthetic monitoring will occur throughout the procedure:

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- ECG rhythm monitor
- O2 saturation using pulse oximeter.
- Blood pressure
- Inspiratory and expiratory O2 and CO2 levels.

10.6. Post treatment care/outpatient discharge criteria

- At least one nurse who is a suitably qualified post anaesthesia care unit nurse will be present in recovery.
- Staff to patient ratio must be 1:1 until the patient's protective reflexes have returned. Refer to [ANZCA PS04\(A\) Position statement on the post anaesthesia care unit.](#)
- Following first stage recovery in the ECT recovery area inpatients will be transferred to the relevant inpatient ward.
- The Consultant Anaesthetist will remain in the ECT Suite until all patients are adequately recovered.
- Refer to the Electroconvulsive Therapy - Recovery and Discharge policy for outpatient discharge criteria.

10.7. Evaluation of treatment

- The Treating Team is responsible for evaluating any side effects and effectiveness of treatment and actively participates in the fortnightly ECT Peer Review meetings.
- Clinical outcomes will be evaluated after six ECT sessions or midway through an ECT course and then again at the conclusion of the course (Please refer to 7.1 for assessment tools).
- The Treating Team will perform a MoCA pre-ECT, post the sixth ECT treatment and after 12 ECT treatments. Three specific versions of the MoCA are available for assessments at the three points in time.
- The Consultant Psychiatrist in charge of ECT and the ECT Coordinator will maintain the ECT Register and the SHIMS ECT database recording the details of the treatment delivered.
- Clinical incidents relating to the practice of ECT will be promptly reported to the Chief Psychiatrist through the Datix Clinical Incident Management System (CIMS).

11. Documentation

- The ECT Treatment Record is completed in SHIMS on the day of treatment and sent for priority scanning into the medical record by the ECT Coordinator.

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- The ECT Course Summary is completed in SHIMS and scanned into the DMR at the completion of a course of ECT.
- The Consultant Psychiatrist in charge of ECT /Head of Service will ensure ECT Statistics are reported to the Chief Psychiatrist monthly on the Mental Health Act Form 13 and for Emergency ECT as required.

12. Compliance monitoring

The ECT Clinical Nurse Specialist together with a Safety and Quality Representative will conduct an annual ECT audit on one or more of the following:

- Documented consent
- ECT Nursing handover
- ECT treatment
- Side effects and complications
- Basic treatment outcomes
- Patient and family education activities
- Patient satisfaction.

The Nurse Unit Manager and the ECT Clinical Nurse Specialist is responsible for ECT and will monitor compliance with this policy.

Pharmacy may complete adhoc audits related to the medication management of patients receiving ECT, all audits will be reviewed at the ECT Meeting.

13. Related policy documents

WA Health:

- [Clinical Incident Management Policy 2019 \(MP 0122/19\)](#)
- [Consent to Treatment Policy MP 0175/22](#)
- [Consent to Treatment Procedure](#)
- [Chief Psychiatrists Guidelines for the Administration of Electroconvulsive Therapy \(ECT\) in WA 2024](#)
- [Chief Psychiatrist Practice Standards for the Administration of Electroconvulsive therapy \(ECT\) in WA 2025.](#)

SMHS:

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- [Credentialing and defining the scope of clinical practice for medical, nursing and midwifery practitioners and allied health and health science professions \(SMHS:96\)](#)

FSFHG:

- Clinical handover (FSFH-HW-POL-0035).
- Patient Identification (FSFH-HW-POL-0034).
- Preoperative fasting (Adult)(FSFHG-HW-POL-0002)
- Observation and Response Chart (FSFH-HW-POL-0032).
- Electroconvulsive Therapy (ECT) Coordinators Role (FSFH-MENH-POL-0013).
- Electroconvulsive Therapy - Recovery and Discharge (FSFH-MENH-POL-0003).
- Electroconvulsive therapy Registrar role (FSFH-MENH-POL-0015).
- Code Blue - FH
- Resuscitation and Emergency Equipment Content and Checking Layout (Adult) (FSFH-HW-GUI-0010)

Other:

- [Royal Australian New Zealand College of Psychiatrists \(RANZCP\) professional practice guidelines for the administration of electroconvulsive therapy \(ECT\) 2019.](#)

14. Related legislation

- The Western Australian Mental Health Act (MHA) 2014
- [Health Practitioner Regulation National Law \(WA\) Act 2010.](#)

15. Related standards

NSQHS standards:

- Clinical Governance
- Medication safety
- Comprehensive Care
- Communicating for safety
- Recognising and responding to acute deterioration

Chief Psychiatrist Standards for Clinical Care:

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- Assessment
- Consumer and carer involvement in individual care
- Physical health care of mental health consumers

16. Bibliography

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Kellner C. Overview of electroconvulsive therapy (ECT) for adults. UpToDate. Post TW (Ed), UpToDate, Waltham, MA. (Accessed on 01/09/2021)

Weiss A, Hussain S, Ng B, Sarma S, Tiller J, Waite S, Loo C. Royal Australian and New Zealand College of Psychiatrists professional practice guidelines for the administration of electroconvulsive therapy. Aust N Z J Psychiatry. 2019 Jul;53(7):609-623. doi: 10.1177/0004867419839139. Epub 2019 Apr 10. PMID: 30966782.

17. Authorisation

EXECUTIVE SPONSOR: Service Director, Service 5					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Authorised By	Revision due
1	07/2015	CNS MH	FH Clinical Governance Committee	Policy & Legislative Compliance Committee	06/2019
2	10/2019	ECT Coordinator	NMPGC	FSFHG Policy Committee	10/2021
3	06/2021	CNS, Improvement	S5 SQRC	SQR Committee, Service 5	06/2023
4	03/2023	CNS, Improvement	NMPGC ECT User Group DTC	SQR Committee, Service 5	03/2026
5	07/2024	CNS, Improvement		SQR Committee, Service 5	08/2027
6.0	07/2025	ECT Coordinator	NMPGC ECT User Group DTC	FSFH MH Clinical Governance Sub Committee	08/2027

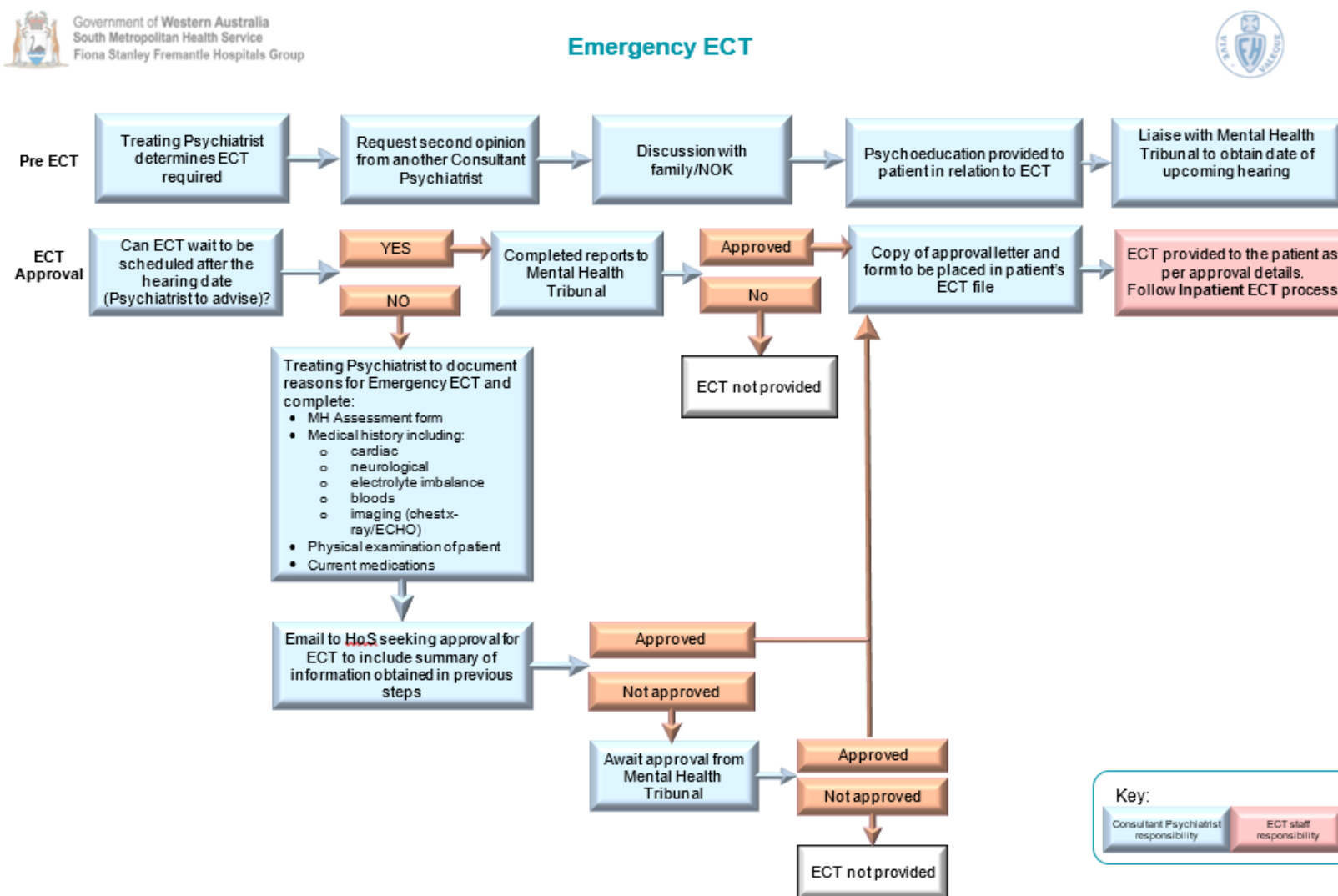
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18. Appendices

18.1. Appendix 1 – Emergency ECT

Emergency ECT



18.2. Appendix 2: Specific information on management of common classes of medications during ECT

(adapted from: Oon H, Schwartz D. Medications and electroconvulsive therapy. Graylands Hospital Drug Bulletin. 2018 June; 25(1): 1-9)

These recommendations are to be used as a guide only, clinical judgement and reasoning will supersede the recommendations in this procedure.

Medication/Class	Risk with ECT Treatment	Recommended Action
Anticonvulsants	Potentially reduce efficacy by increasing seizure threshold	If used as mood stabiliser – cease medication if possible. Withhold dose from the night prior to ECT and can be administered after ECT treatment as soon as practical. Lamotrigine may be considered an exception and continued, due to its lesser effect on seizure induction and duration, difficulty in titrating dose, and risks associated with fluctuating plasma levels. If seizures are impacted, consider reducing the dose, or altering the timing of the dose to post-ECT (if being used once daily). If used for epilepsy – continue medication and withhold dose from the night before ECT
Benzodiazepines	Potentially reduce efficacy by shortening seizure duration	Cease if possible. If benzodiazepines cannot be discontinued, convert benzodiazepine with shorter half-life such as lorazepam and withhold from 2pm the day prior to ECT. Use non-benzodiazepines such as quetiapine for sleep/anxiolytic. Zolpidem/zopiclone can be used for insomnia due to their short half-life. Consider flumazenil pre-treatment prior to ECT if benzodiazepine cannot be discontinued or converted to alternative benzodiazepines with a shorter half-life. If benzodiazepine withdrawal symptoms occur within an hour post ECT, intravenous midazolam can be administered following ECT treatments.

Medication/Class	Risk with ECT Treatment	Recommended Action
Antipsychotics	Antipsychotics (particularly clozapine and phenothiazines) lower the seizure threshold and would be expected to lead to seizures at lower ECT doses.	Combination of antipsychotics and ECT appears to be generally well tolerated and effective. Continue treatment with antipsychotic unless issues are encountered. Note: Although generally considered safe, Clozapine may carry a risk of prolonged or unwanted seizures, protracted delirium or serious hypersalivation. If issues are encountered, consider reducing the dose, or withholding for 24hrs pre-ECT.
Antidepressants	Potential of increase in seizure duration. Risk to clinical impact is minimal. Potential risk of asystole and cognitive adverse effects with venlafaxine doses $\geq 300\text{mg}$ Potential increase risk of cardiac arrhythmias with tricyclic antidepressants (TCA), especially in the elderly population and individuals with cardiac disease.	Continue treatment for all antidepressants unless issues are encountered except for: • Venlafaxine: Dose reduction to $\leq 225\text{mg}$ • Desvenlafaxine: Dose reduction to $\leq 150\text{mg}$ Consider risk of benefit of continuing TCAs, given that there are evidence of safety and improvement of cognitive outcomes with nortriptyline.
Lithium	Risk of cognitive disturbances, prolonged apnoea and spontaneous seizures. Recent evidence demonstrated safety with concurrent administration. Theoretical interaction with suxamethonium prolonging its action.	Reduce lithium level to the lower end of the therapeutic range prior to ECT and withhold dose on night prior to ECT
Stimulant Medications	Potential increased risk of hypertension and tachycardia during clonic phase if given pre-ECT.	Withhold on the morning of ECT. Doses may be given post ECT. Note: administering long-acting medications too late in the day may cause insomnia.
Antihypertensives	Risk of hypertension during clonic phase of ECT if regular dose not administered prior	Administer dose with small sips of water prior to ECT with the exception of diuretics
Antianginals (beta-blockers, nitrates)	Risk of arrhythmias	Administer dose with small sips of water prior to ECT

Medication/Class	Risk with ECT Treatment	Recommended Action
Anticoagulants Antiplatelets	Possible risk of intracerebral haemorrhage	Generally safe to continue anticoagulant and antiplatelet agents during a course of ECT as the risk of stopping these medications and running into cardiac issues would appear to vastly outweigh the tiny risk of intracerebral bleeding
Diuretics	Risk of incontinence during seizure	Withhold dose on morning of ECT and administer post ECT
Asthma medication (preventers)	Risk of asthma exacerbation during ECT if not administered	Administer dose prior to ECT with the exception of theophylline.
Theophylline	Risk of prolonged seizure or status epilepticus	Cease prior to ECT if possible or maintain at lowest therapeutic dose if ceasing medication not appropriate.
Antidiabetic medications	Hypoglycaemia as patient fasting from night before ECT	Withhold all oral antidiabetic medications, short-acting insulin, intermediate-acting insulin and daily GLP-1 analogues (e.g. liraglutide) on the morning of ECT and administer post ECT when patient can start eating. If basal insulin (e.g. glargine) is administered: In the evening - Continue usual dose night prior to ECT. In the morning - Administer 50% of the usual dose the morning of the procedure and administer 50% of the usual dose once the patient starts eating post ECT.
Gastrointestinal Medications	Risk of reflux and potential aspiration during ECT if not administered	Administer dose with small sips of water prior to ECT

11.3 Appendix 3. Titration Schedule for right unilateral ultra brief placement

Threshold Settings			Treatment Settings		
Threshold dosage (TD) %	PW	Charge (mC)	Treatment dosage 5-6 x TD %	PW	Charge (mC)
2 (Female <55 yrs)	0.3	10	10	0.3	50
4 (Female >55yrs) (Male <55yrs)	0.3	20	25	0.3	126
6 (Male >55yrs)	0.3	30	35	0.3	176
8	0.3	40	50	0.3	252
15	0.3	76	90	0.4	454
25	0.3	126	150	0.5	756
35	0.3	176	200	0.6	1008
50	0.3	252	200	0.75	1008
75	0.3	378	200	0.75	1008

- First session – to aim to detect seizure threshold.
- Subsequent sessions – To administer treatment dose. If seizure quality is poor move to next level as per right column.

11.4 Appendix 4 Titration schedule for bilateral placement

Threshold Settings			Treatment Settings		
Threshold dosage (TD) %	PW (ms)	Charge(mC)	Treatment dosage 1.5 x TD %	PW (ms)	(mC)
10 (Female <55 yrs)	0.5	50	15	0.5	75
15 (Female >55yrs) (Male <55yrs)	0.5	75	23	0.5	115
20 (Male >55yrs)	0.5	100	30	0.5	151
25	0.5	126	40	0.5	201
30	0.5	151	45	0.5	226
35	0.5	176	55	0.5	277
40	0.5	201	60	0.5	302
45	0.5	226	70	0.5	352
50	0.5	252	75	0.5	378

- First session – to aim to detect seizure threshold.
- Subsequent sessions – to administer treatment dose. If seizure quality poor move to the next level as per right column.